# Relationships, Social Connection, and Mental Wellbeing

A practical, evidence-informed learning chapter for individuals, families, students, and communities in Rwanda.

### Document status

Version 1.0 | 11 November 2026 | English. This is public-facing educational draft content, not a clinical assessment or personal treatment plan. Professional clinical, safeguarding, accessibility, and Kinyarwanda/French reviews are pending.

### Learning outcomes

- Describe how relationships and social connection can affect mental wellbeing.
- Distinguish loneliness, social isolation, solitude, and supportive connection.
- Recognize respect, reciprocity, healthy boundaries, and concerning interpersonal patterns.
- Practice communication, conflict resolution, and safe help-seeking skills.
- Identify when loneliness or relationship stress warrants professional support.

### How to use this chapter

Read the sections in order, or choose a topic relevant to your situation. All scenarios are fictional. Exercises are optional; skip any that feel unsafe, intrusive, or emotionally overwhelming. For children, tailor activities with appropriate adult or professional guidance. This chapter does not require disclosure of personal experiences.

## 01. Why relationships matter for mental health

People develop and maintain wellbeing through many forms of connection: friendships, family ties, classmates, colleagues, neighbours, mentors, peers, and chosen communities. Supportive relationships may offer comfort during setbacks, practical assistance, recognition, shared enjoyment, and a sense of belonging. They are one among many influences on mental health, alongside biology, life experiences, economic security, and access to healthcare [1-3].

Social connection is not measured only by the number of people in someone’s contacts. A person may have many acquaintances but few relationships in which they feel safe and understood. Another person may have a small circle and feel well supported. Relationship quality, reliability, and personal

choice matter.

Connection can protect wellbeing without guaranteeing that a person will never experience depression, anxiety, or another mental health condition. Similarly, being socially isolated does not prove someone has a disorder. [1-3]

# 02. Understanding social connection, loneliness, isolation, and solitude

## Social connection

Social connection describes the structure, functions, and quality of how people relate to others. Structure includes who is in a person's network and how often they interact. Function includes the type of emotional or practical support exchanged. Quality concerns whether interactions feel supportive, trustworthy, strained, or harmful. This three-part explanation follows the WHO social-connection framework [1].

## Loneliness

Loneliness is a distressing feeling that occurs when a person experiences a gap between the relationships they want and those they believe they have. A person can feel lonely in a crowded classroom, busy workplace, or household. Loneliness is not a character flaw or proof of unpopularity [1].

## Social isolation

Social isolation refers to having relatively few relationships, social roles, or interactions. It may result from geographic distance, disability-related barriers, bereavement, discrimination, relocation, low income, or other circumstances. Someone can be isolated without feeling lonely, although the two often overlap [1].

## Solitude

Solitude is time spent alone, sometimes intentionally and comfortably. Quiet time for rest, prayer, creative work, or reflection can be valuable. Being alone is not automatically unhealthy. What matters is whether the person can access the level and quality of connection they need.

### Reflection

Which word best describes your current experience: connected, lonely, isolated, enjoying solitude, or unsure? There is no right answer. You may experience more than one at the same time.

# 03. Healthy relationships: what to look for

A healthy relationship is not a relationship without disagreements. It is one in which people can disagree, grow, and make choices while preserving one another's dignity. Healthy relationships may include:

- Respect: each person’s needs, privacy, identity, and decisions are taken seriously.
- Trust: promises are generally kept; questions can be discussed without intimidation.
- Mutuality: both parties have room to speak, give, receive, and make choices; equal contribution is not required every day.
- Boundaries: people can say no, request time apart, and change their mind.
- Support: care is offered without making the other person responsible for every emotion or problem.
- Repair: after mistakes, people can acknowledge harm, take responsibility, and change behaviour.

Family, friendship, romantic, and professional relationships have different expectations. The same principle applies across them: caring for someone does not authorize monitoring, humiliating, isolating, or coercing them.

## 04. Social support: emotional, practical, informational, and belonging

People need different kinds of support at different times. Emotional support involves empathy and being listened to. Practical support may include help with transportation, childcare, or sharing a task. Informational support involves reliable guidance or help finding services. Belonging comes from being part of a group in which a person feels accepted [2,3].

A student worried about examinations may benefit from a peer who listens, a tutor who helps organise revision, and a trusted adult who notices exhaustion. No single person has to meet all those needs. Developing more than one safe connection can reduce dependence on an individual relationship.

Offering support should not become pressure. Ask what someone wants before giving advice, involving others, or sharing a private situation.

## 05. Why making and maintaining connections can be difficult

Challenges such as moving to a new district, unemployment, heavy school workloads, disability-related access barriers, caregiving, bereavement, social anxiety, experiences of discrimination, or limited money for transport can make maintaining relationships difficult. This is not evidence that a person is unwilling or incapable of friendship.

Some people also need more time alone than others. The goal is not to force sociability or make everyone attend large gatherings. It is to support meaningful relationships that fit individual preferences and safety.

When a person has been hurt or controlled in a relationship, rebuilding trust may require time and professional support. Encouraging immediate reconnection with unsafe individuals can increase risk.

# 06. Listening and communicating with care

Communication is not just about speaking clearly. It also involves checking what someone means, allowing silence, and noticing whether the conversation feels safe. Helpful skills can be practised gradually.

## A four-step conversation method

- Choose a suitable moment. Ask whether the other person has time and privacy to talk.
- Describe what you observed without making an accusation: “I noticed we have barely spoken this week.”
- Explain your feeling or need using “I” language: “I miss our conversations and would like to reconnect.”
- Invite a response: “How has the week been for you?” Listen before deciding what to do next.

An “I” statement can make concerns easier to discuss, but it is not a guaranteed way to resolve conflict. If the other person threatens or intimidates you, communication techniques are not a substitute for safety planning.

## Active listening

Active listening involves giving attention, avoiding unnecessary interruption, reflecting the meaning of what someone says, and asking permission before offering advice. A supportive reply may sound like: “That sounds exhausting. Would you prefer that I listen or help you think about options?”

# 07. Interpersonal conflict and healthy repair

Disagreements can arise from expectations, stress, financial strain, misunderstandings, or different priorities. Constructive conflict management includes slowing down, focusing on a specific issue, allowing each person a voice, agreeing on a realistic next step, and revisiting the matter when calm.

For instance, two housemates may disagree over shared chores. They could identify the tasks, discuss what each can realistically manage, write down a fair arrangement, and review it after a week. The goal is not winning an argument but developing a workable agreement.

### Safety distinction

Ordinary disagreement is not the same as intimidation, coercive control, stalking, threats, or violence. If a person feels unsafe, avoid encouraging joint confrontation, mediation, or couples exercises as a default. Seek appropriate specialist assistance instead.

## 08. Emotional boundaries and personal autonomy

Boundaries are limits that protect time, privacy, emotional energy, and personal choices. A boundary is not a punishment; it describes what a person can accept and what they will do to care for themselves. Examples include choosing when to answer messages, deciding what information to share, asking before discussing a sensitive topic, and refusing to lend money one cannot afford to lose.

A respectful boundary might be: “I cannot talk about this during work. I can speak after 6 p.m.” Another might be: “I am not ready to discuss that experience.” Setting a boundary does not guarantee another person will respect it. In unsafe relationships, personal safety should take priority over explaining a boundary repeatedly.

Emotional support does not mean becoming someone’s clinician, remaining available every hour, or keeping dangerous secrets. It is appropriate to seek extra help when a problem exceeds what a friend or family member can safely manage.

## 09. Loneliness and its effects on wellbeing

Prolonged loneliness and isolation are associated with poorer mental and physical health, although the relationship is complex and not all individuals are affected in the same way [1-4]. Loneliness may involve sadness, restlessness, reduced motivation, difficulty trusting others, or the sense that one is disconnected even while surrounded by people.

Noticing loneliness can be a useful signal that a need for connection is unmet. It should not be treated as a diagnosis. A person whose loneliness occurs alongside persistent low mood, anxiety, sleep disturbance, or substantial impairment may benefit from professional assessment.

### Gentle steps toward connection

- Identify one person or community setting that has previously felt reasonably safe.
- Choose a small action, such as a brief greeting, a message, or attending one group activity.
- Prefer shared interests over pressure to disclose private feelings.
- Repeat contact at a manageable pace; friendships often develop through time.
- If a group does not feel welcoming or safe, it is acceptable to try another setting.

## 10. Digital relationships and online wellbeing

Messaging apps, online learning groups, and social platforms can help people stay connected across long distances. They can also create pressure to respond immediately, comparisons with others, misunderstandings, or exposure to harassment. The quality and safety of an interaction matter more than whether it happens online or in person.

Useful digital boundaries include asking before sharing someone else’s photo or story, respecting delayed replies, choosing privacy settings thoughtfully, and avoiding arguments in public comment sections when a private conversation would be safer. Digital access may not be private for someone whose device is monitored.

If someone is being threatened or monitored, changing settings or blocking a contact might alert the person causing harm. Specialist digital safety support may be needed before taking action.

## 11. Families, friends, peers, and caregivers

Families and caregivers can support wellbeing through reliable attention, predictable care, encouragement, and help during difficulties. Friends and peers may provide acceptance and shared experiences. Teachers, mentors, health workers, and community members may also become valuable sources of support.

However, not every family or community relationship is safe. People should not be encouraged to disclose private mental health concerns to relatives, religious leaders, or peers without considering trust, confidentiality, and personal choice.

For adolescents, safe relationships with trusted adults are important. Adults should listen respectfully, avoid dismissing concerns as “just a phase,” and seek appropriate professional or safeguarding assistance when serious risks arise [3,5].

## 12. Community belonging in Rwanda

In Rwanda, possible opportunities for social connection may include school clubs, youth groups, sports teams, community activities, volunteer initiatives, faith communities, study circles, neighbourhood associations, and peer-support programmes. These are illustrative settings, not guaranteed services or endorsements.

Participation should remain voluntary. Activities should be inclusive of people with disabilities, individuals living far from community centres, people with limited connectivity, and those who may not feel safe in some groups. Privacy and protection from stigma are especially important when mental health is discussed.

Community programmes can support connection by providing accessible meeting places, predictable schedules, welcoming facilitators, and a way to raise concerns safely [1,2].

## 13. Supporting someone who feels lonely or distressed

Begin by asking how the person would like to be supported. Offer specific, practical options instead of making broad promises. For example: “Would you like me to check in this weekend?” or “Would it help to walk together after class?” Respect a no and avoid turning an invitation into pressure.

Do not assume that frequent contact is always best. Some people need time, rest, or privacy. If a person speaks about hopelessness, self-harm, or suicide, take it seriously and seek appropriate urgent professional support. Do not promise to keep an immediate safety risk secret.

## 14. When professional support is appropriate

Consider consultation with a qualified healthcare or mental health professional when loneliness, social withdrawal, or interpersonal stress becomes persistent, overwhelming, or interferes with sleep, school, employment, or daily responsibilities; when anxiety or depression symptoms are present; or when repeated conflict or trauma makes it difficult to feel safe.

Urgent assistance is needed when there is immediate danger, threats of serious violence, or concern that someone may act on thoughts of suicide or self-harm. Seek available emergency medical assistance or the nearest appropriate emergency facility. Specific Rwanda contacts and operating hours must be confirmed before publication of this chapter.

## 15. Fictional learning scenarios

### Scenario A — A first-year university student in Kigali

A student moved from Musanze for university. Although they speak with many classmates, they feel they have no one to rely on. They decide to attend a small study group twice and message a former school friend. Discussion: Why can someone be lonely around other people? What makes a manageable first step?

### Scenario B — A parent balancing work and caregiving

A parent with long working hours has little opportunity to meet friends. Instead of committing to several activities, they arrange a short weekly call with a trusted person and ask whether a neighbour can occasionally help with a practical task. Discussion: What barriers make connection difficult? How can support include practical help?

### Scenario C — Two friends disagree

One friend feels ignored when replies arrive late. The other explains they often share a phone and have limited data. They agree not to interpret slow replies as rejection and decide on a regular time to catch up. Discussion: How did assumptions influence the disagreement?

### Scenario D — Pressure within a relationship

A person’s partner demands their phone password, discourages friendships, and becomes angry when they attend a community event. A trusted mentor suggests individual specialist support rather than an immediate confrontation. Discussion: Which behaviours go beyond ordinary disagreement, and why does safety come first?

### Scenario E — A community sports group

A newcomer wants to join a local sports team but worries about not fitting in. The organizer explains expectations kindly, allows the person to observe initially, and provides an accessible way to ask questions. Discussion: How does a welcoming environment strengthen belonging?

# 16. Guided educational activities

## Activity 1 — My support map (10–15 minutes)

On paper, draw three circles labelled “People I trust,” “Places or groups I may choose,” and “Professional services if needed.” Add only contacts you genuinely consider safe. Do not include identifying information if someone else may see the page. A blank circle is not a failure; it identifies a possible area to build gradually.

## Activity 2 — Connection without pressure (7 days)

Choose a low-risk, realistic action: send a greeting, attend one community activity, sit with a classmate at lunch, or reconnect with a trustworthy acquaintance. Afterward, note how comfortable it felt, whether the interaction was respectful, and whether you want to repeat it. Stop or adapt the exercise if it increases distress or risk.

## Activity 3 — Listening practice

With a willing and trusted partner, take turns speaking about a neutral topic for two minutes. The listener does not interrupt and asks one clarifying question. Then switch roles. Reflect on the difference between listening and immediately solving a problem. Do not use sensitive trauma disclosures for this exercise.

## Activity 4 — Boundary sentence

Write a simple statement that describes a limit without blaming another person: “I need some time before discussing this” or “I prefer not to share my account password.” You do not need to deliver it to someone if that would be unsafe.

## Activity 5 — Community inclusion audit

Think of a school club, sports group, or community event. Ask: Can people attend with limited money or transport? Are newcomers welcomed? Can someone leave without pressure? Are disability needs considered? Is private information protected? Identify one change that could make the activity safer and more inclusive.

# 17. Knowledge assessment

1. Can a person feel lonely despite having many social contacts? Yes. Loneliness is about perceived unmet connection needs, not only the number of contacts.

2. Are solitude and loneliness identical? No. Solitude may be chosen and restorative, while loneliness involves distress.

3. Name two elements of a healthy relationship. Examples: respect, trust, boundaries, mutuality, support, or repair after mistakes.

4. What is one example of practical social support? Help with transportation, childcare, or completing a necessary task.

5. What does active listening involve? Attention, respectful questions, reflection, and not interrupting unnecessarily.

6. Does an argument always mean abuse? No. However, threats, coercion, intimidation, or controlling patterns warrant safety attention.

7. Is sharing a friend’s personal story online acceptable without asking? No. Seek informed permission and respect privacy.

8. Must someone attend large social gatherings to be mentally healthy? No. Connection needs and preferences vary.

9. What should you do if setting a boundary could trigger danger? Prioritise safety and seek appropriate professional or specialist guidance.

10. When might loneliness warrant professional support? When persistent or accompanied by significant distress, impaired functioning, or other concerning symptoms.

## 18. Summary and key messages

- Supportive relationships contribute to wellbeing, but people need different types and amounts of connection.
- Loneliness is a feeling; social isolation refers to limited interactions; solitude can be chosen.
- Quality, respect, boundaries, and safety matter as much as contact frequency.
- Small, repeatable, voluntary social actions may be more useful than pressured participation.
- A safe relationship allows disagreement without intimidation or coercion.
- Professional care is appropriate when distress persists or functioning is affected.

## 19. Glossary

Social connection: Relationships and interactions considered in terms of structure, support function, and quality.

Loneliness: Distress caused by a perceived gap between desired and existing connections.

Social isolation: Relatively few social relationships or interactions.

Emotional boundary: A limit around emotional availability, disclosure, or personal responsibility.

Active listening: Attentive, respectful listening aimed at understanding rather than immediately judging or fixing.

Coercive control: A pattern of intimidation or restrictions used to dominate another person.

Belonging: Feeling accepted and valued as part of a relationship or group.

## 20. References and evidence notes

[1] WHO. Social connection. Questions and answers, 30 June 2025. https://www.who.int/news-room/questions-and-answers/item/social-connection

[2] WHO Commission on Social Connection. From loneliness to social connection: charting a path to healthier societies. 2025. https://www.who.int/publications/i/item/978240112360/

[3] US Centers for Disease Control and Prevention. Social Connection. 15 May 2024. https://www.cdc.gov/social-connectedness/about/

[4] CDC. Health Effects of Social Isolation and Loneliness. 15 May 2024. https://www.cdc.gov/social-connectedness/risk-factors/index.html

[5] CDC. Improving Social Connectedness. 15 May 2024. https://www.cdc.gov/social-connectedness/improving/index.html

[6] WHO. Mental health: strengthening our response. 2026. https://www.who.int/news-room/fact-sheets/detail/mental-health-strengthening-our-response

Evidence note: The chapter paraphrases official source guidance and adds original, fictional scenarios and educational exercises. These exercises are practical teaching examples; they have not been independently tested as a clinical intervention. No local referral contact is asserted as verified in this version.

## Publication review checklist

Before public release: (1) clinical review; (2) safeguarding and domestic-violence safety review; (3) fact-check of references and Rwanda-specific services; (4) plain-language and accessibility review; (5) culturally appropriate Kinyarwanda/French translations; (6) version control and scheduled review date.